

COOK COUNTY BEHAVIORAL HEALTH SERVICES

Division of Child and Adolescent Psychiatry

2720 S. California Ave., Suite 200, Chicago, IL 60608

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OUTPATIENT PSYCHIATRY NOTE

Patient: Marcus D. Whitfield MRN: PED-2025-04471

DOB: 03/03/2016 Age: 9 years, 10 months Sex: Male

Date of Visit: 01/10/2026 Visit Type: Follow-up

Attending: Kwame Osei, MD — Child and Adolescent Psychiatry

Accompanied by: Caregiver (foster parent at prior placement)

CHIEF COMPLAINT

Follow-up for behavioral management. Caregiver reports worsening irritability, aggression toward peers, and nighttime sleep disruption over the past 6 to 8 weeks.

HISTORY OF PRESENT ILLNESS

Marcus is a 9-year-old male with Autism Spectrum Disorder Level 2, ADHD Combined Type, Post-traumatic Stress Disorder, and Fetal Alcohol Spectrum Disorder, seen today for psychiatric follow-up. He is accompanied by his current foster parent, who reports escalating behavioral difficulties including physical aggression toward classmates during unstructured activities, increased verbal outbursts at home, resistance to transitions, and disrupted sleep with frequent nighttime awakenings. These symptoms have worsened over the past 6 to 8 weeks per caregiver report.

Marcus has a history of trauma-related behavioral dysregulation, including three prior psychiatric hospitalizations for self-injurious behavior in the context of placement transitions. His current behavioral presentation is attributed by the caregiver primarily to school stressors and uncertainty regarding his living situation. He has been in three foster placements over the past three years, with the most recent transition approximately 10 months ago.

Caregiver denies current suicidal ideation, self-harm behaviors, or homicidal ideation. She notes that Marcus does engage in repetitive behaviors including hand-flapping and rocking, which have not changed in frequency. Appetite is described as variable with strong food preferences and limited range of accepted foods consistent with sensory processing differences.

PSYCHIATRIC HISTORY

Diagnoses: ASD Level 2 (diagnosed age 4), ADHD Combined Type (diagnosed age 6), PTSD (diagnosed age 7 following second placement change), FASD (diagnosed age 3 per genetic/developmental evaluation). Three prior psychiatric hospitalizations for self-injurious behavior, most recent age 8. No history of substance use.

CURRENT MEDICATIONS

- Risperidone 1.5 mg PO QHS (for irritability/aggression associated with ASD)
- Methylphenidate ER 27 mg PO QAM (ADHD — prescribed by Dr. Liang, Developmental Pediatrics)
- Clonidine 0.1 mg PO QHS (sleep initiation, hyperarousal)
- Sertraline — discontinued approximately 3 months ago per caregiver; previously 50 mg QHS for anxiety/PTSD symptoms. Caregiver states it was stopped due to perceived lack of benefit.
- Budesonide/Formoterol inhaler BID, Albuterol MDI PRN (asthma — managed by PCP)
- Ferrous sulfate 325 mg daily (anemia — managed by GI)
- Melatonin 3 mg QHS (OTC, caregiver-administered)

MENTAL STATUS EXAMINATION

Appearance: Age-appropriate male, thin, casually dressed, fair hygiene. Motor activity: intermittent hand-flapping, difficulty remaining seated, fidgeting with zipper. Eye contact: limited and fleeting. Speech: low volume, short phrases, occasional echolalia. Mood: 'I don't know' (patient); 'angry and sad' (per caregiver). Affect: restricted range, congruent with described mood. Thought process: concrete, goal-directed with prompting. Thought content: denies SI/HI when asked directly; caregiver confirms no current self-harm. No delusions or hallucinations elicited. Cognition: oriented to person and place, not reliably to date. Insight: limited. Judgment: impaired for age.

ASSESSMENT

1. ASD Level 2 with associated irritability and aggression — worsening per caregiver report, likely multifactorial (school stressors, placement instability, disrupted routines). Current risperidone dose at 1.5 mg QHS provides partial benefit. Recommending dose increase to 2 mg QHS given worsening target symptoms.
2. ADHD, Combined Type — partially managed on methylphenidate ER 27 mg. Deferring stimulant adjustment to Dr. Liang (Developmental Pediatrics).
3. PTSD — sertraline was discontinued approximately 3 months ago. Given the current behavioral presentation and history of trauma-related dysregulation, the role of SSRI therapy should be reassessed at the next visit if behavioral interventions do not result in meaningful improvement.
4. Sleep disruption — multifactorial. Clonidine and melatonin continued. Rule out medical contributors (caregiver describes loud snoring and occasional pauses in breathing during sleep — defer to PCP for further evaluation).

PLAN

- Continue risperidone 1.5 mg QHS for now. Increase to 2 mg QHS at next visit given worsening aggression and irritability.
- Continue clonidine 0.1 mg QHS.
- Continue melatonin 3 mg QHS.
- Defer ADHD medication management to Dr. Liang.
- Discussed sleep concerns with caregiver, including observed snoring and possible apneic episodes. Recommended PCP evaluate for obstructive sleep apnea given ASD, FASD, and reported symptoms.
- Behavioral therapy referral renewed. Caregiver to contact Cook County Behavioral Health intake for ABA and trauma-focused CBT services.
- Follow-up in 8 weeks or sooner if behavioral escalation requires urgent reassessment.

Kwame Osei, MD

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Electronically signed: 01/10/2026 at 16:22 CST

Dictated and finalized on 01/10/2026